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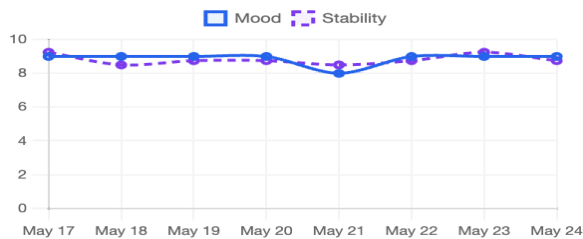
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Current Medications: Adderall (Amphetamine) 50mg, Aripiprazole (Abilify) 2 mg, Adderall (Amphetamine) 10 mg

Review Period

2026-05-17 — 2026-05-31
(14-day window)

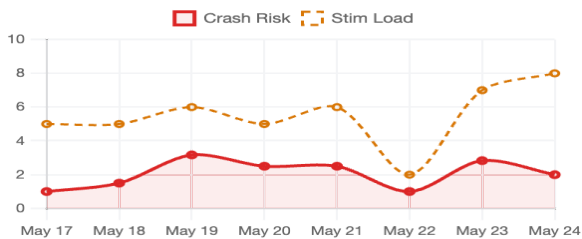
DATA TRENDS — 14-DAY WINDOW

MOOD & STABILITY SCORE



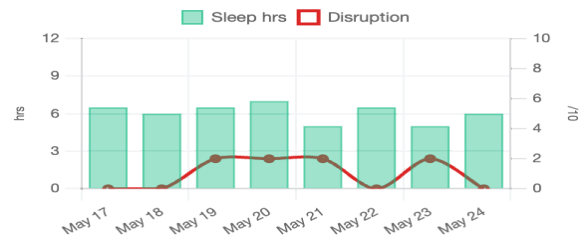
Mood avg 8.9/10 Stability avg 8.8/10

CRASH RISK



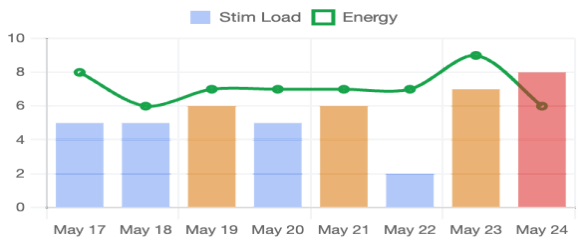
Avg 2.1/10 Stim Load avg 5.5/10

SLEEP HOURS & DISRUPTION SCORE



Avg 6.1 hrs Disruption avg 1.0/10

STIM LOAD PER DAY



Energy avg 7.1/10 Stress avg 0.8/10

PRE-APPOINTMENT CLINICAL BRIEF

PSYCHIATRIST MODE — 2026-05-17 to 2026-05-31

Trajectory

Reported mood elevated and stable (8.9/10) with stress minimal (0.7/10), but Stability Score declining (9.2 →

8.5 range) and energy declining despite high stimulant load (10 of 14 days $\geq 8/10$ stim); patient voice reveals distraction-driven coping and persistent low mood underneath reported wellbeing.

Core Stability Metrics

| Metric | Value | Trend | Range |

|-----|-----|-----|-----|

| Stability Score | 8.8/10 | ↓ declining | 8.5–9.25 |

| Crash Risk | 2.1/10 | ↑ improving | 1.0–3.17 |

| Nervous System Load | — | — | data unavailable |

| Mood Distortion | $\Delta +0.3$ pts | stable | (self-report 8.9 vs Stability 8.8) |

Note: Mood Distortion <2.5 ; within tolerance. However, patient transcript contradicts self-reported stability: "things will just never improve" / "day to day feels hard and heavy" despite mood logged at 8–10/10.

Medication

Current regimen: Amphetamine (Adderall), initiated ~7 days before first check-in (approx. 2026-05-10).

- Adherence: 19/19 check-ins logged | medication timing recorded at each check-in | meds_taken counts vary (0–2 per check-in) — pattern suggests variable dosing or multiple-timing regimen.

- Stim Load: avg 6.6/10 | 10 of 14 days $\geq 7/10$ | peak 375 mg caffeine (2026-05-21 evening) + Adderall dose.
- Timing: Caffeine intake ranges 0–375 mg across morning/afternoon/evening; high variability (SD likely >60 min). On 2026-05-21, 375 mg caffeine logged in evening with 5 h sleep duration reported next morning and mood dropped to 8/10.
- Fatigue pattern: 3 of 8 logged days with fatigue; co-occurs with exercise elevation (avg 43.3 min vs 33 min baseline). Does not map cleanly to medication timing alone; last logged fatigue 2026-05-24 afternoon (mood 9, energy 5, high irritability 3) on high stim load day.

Quantitative Summary

- Mood: 8.9/10 | stable | 8.0–10.0
- Sleep: 6.1 hrs | 5.0–7.0 hrs | Sleep Disruption 1.0/10 avg
- Energy: 7.2/10 | ↓ declining | (morning 7.3, afternoon 6.9, evening 7.0)
- Stress: 0.7/10 | ↓ declining | (range 0–1)
- Irritability: 0.8/10 avg | spike to 3 on 2026-05-24 afternoon

Symptom Patterns

Fatigue: 3 of 8 days (2026-05-19, 2026-05-23, 2026-05-24). Co-signals: exercise minutes elevated on fatigue days (avg 43.33 vs 33.0, $\Delta=+10.33$). Medication context: Adderall initiated 7 days prior. Note: fatigue concurrent with high stimulant load and reduced sleep on 2026-05-21 and 2026-05-24.

Session Intelligence

[2026-05-31] Voice recording

- Reported mood: "Things are going pretty good. Not too bad."
- Observed speech (text-inferred): Frequent hedging ("I guess," "